

14 DAY WAIT – URGENT REFERRAL

ROYAL STOKE UNIVERSITY HOSPITAL

**OESOPHAGUS
AND STOMACH**

General Enquiries:

Tel 01782 671117

Clinical Enquiries:

Tel 01782 675703

**ALL REFERRALS TO BE SENT VIA RAS (REFERRAL ASSESSMENT SERVICE) YOU ARE NO
LONGER REQUIRED TO EMAIL/FAX UBRN**

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July 2017

PATIENT DETAILS				GP DETAILS			
Name:	Given Name Surname			Name:	Registered GP Title Registered GP Forenames Registered GP Surname		
Address:	Home Full Address (single line)			Address:	Organisation Name, Organisation Full Address (single line)		
NHS Number:	NHS Number			Phone No:	Organisation Telephone Number		
Hospital Number:	Hospital Number			Email Address:	Organisation E-mail Address		
Date of Birth:	Date of Birth			Name of referrer:			
Gender:	Gender(full)			Decision to refer date:		Short date letter merged	
Interpreter required:		☐ Yes ☐ No		Sign Language required:		☐ Yes ☐ No	
Language:		Main Language					
Contact No (next 48 hrs):	Home:	Patient Home Telephone	Work:	Patient Work Telephone	Mobile:	Patient Mobile Telephone	

REASON FOR REFERRAL (please indicate A or B as appropriate)**A: URGENT ENDOSCOPY (<2 weeks)**☐ Dysphagia – clear description of food sticking following initiation of swallow*Please complete the Edinburgh Dysphagia Score below for all dysphagia referrals. This is recommended by the BSG to help endoscopy units triage endoscopy referrals to the most likely to have a cancer diagnosis.***Please record the points against each item below and the total EDS score**

A	Points	B	Points	D	Points	F	Points
Age		Gender		Dysphagia localizes to neck		Duration of symptoms > 6mths	
18-39 ☐	0	Male ☐	0	Yes ☐	-2	Yes ☐	-1.5
40-49 ☐	4	Female ☐	-1	No ☐	0	No ☐	0
50-59 ☐	5						
60-69 ☐	6						
70-79 ☐	7						
80-89 ☐	8						
90-99 ☐	9						

A	B	C	D	E	F

Total EDS score = A+B+C+D+E+F

An Edinburgh Dysphagia Score of ≥ 3.5 will proceed to urgent endoscopy if fit.An Edinburgh Dysphagia Score of < 3.5 will be listed for endoscopy on a non-urgent basis or may be triaged for further telephone assessment and non-endoscopic investigations such as barium or CT imaging

If an EDS score cannot be completed in Primary Care, please confirm reason and please supply as much information as possible in relation to questions C – F to inform this triage process.

If the EDS score is not included in the referral this may lead to a delay in access to diagnostic tests

Name: Given Name Surname NHS No: NHS Number

UHNM - Upper GI 2ww Referral Proforma

Final V5 UHNM (April 2023)

Aged 55 and over with weight loss and any of the new following:

- ☐ upper abdominal pain
☐ reflux
☐ dyspepsia

(if dyspepsia without weight loss to refer to NICE CG184: GORD and dyspepsia in adults)

B: FAST TRACK CLINIC APPOINTMENT (<2WEEKS)

- ☐ Upper abdominal mass consistent with stomach cancer

Clinical Details (History / Examination)

Drugs ☐ Aspirin ☐ NSAID ☐ Warfarin ☐ PPI/H₂RA ☐ Clopidogrel
☐ Newer Antiplatelet ☐ Newer Anticoagulant

Medical History ☐ Diabetes ☐ Insulin ☐ Non-insulin
☐ GI Surgery Details:

Previous investigations	Gastroscopy	When	Result
	Barium Meal	When	Result
	Helicobacter	When	Result
	Ultrasound	When	Result

Family History / Any other information

Accessibility Needs:

- ☐ Wheelchair access
☐ Deaf
☐ Registered blind
☐ Learning Disability
☐ Other disability needing consideration
☐ Accompanied by carer

WHO Performance Status:

- ☐ 0 Fully active
☐ 1 Able to carry out light work
☐ 2 Up and about greater than 50% of waking time
☐ 3 Confined to bed/chair for greater than 50%
☐ 4 Confined to bed/chair 100%

☐ I confirm that I have discussed the possibility that the diagnosis may be cancer and they may wish to take a family member or significant other to the initial appointment.

☐ I confirm that I have requested FBC, U+E, LFT and GFR (& INR if jaundiced or liver lesions)

☐ I confirm that the patient is available to attend outpatients within the next 14 days

If patient is not available for the next 2 weeks, and aware of nature of referral, consider deferring the referral and seeing the patient again to reassess symptoms

Reference: NICE Guideline recommendations for Suspected cancer: recognition and referral, July 2017 and West Midlands Cancer Alliance UGI 2WW referral form Feb 2022

Relevant Test Results (where available)

Full Blood Count (FBC)	See table below	Liver Function Test	See table below
U&E	See table below	eGFR	Single Code Entry: GFR (glomerular filtration rate) calculated by abbreviated Modification of Diet in Renal Disease Study Group calculation...

Name: Given Name Date of Birth NHS No: NHS Number

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Smoking Status	SEE BELOW	Alcohol Status	SEE BELOW
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Smoking Status
Smoking

Alcohol Consumption
Alcohol Consumption

Allergies
Allergies

Medication:
Medication

Relevant Medical History
Problems

Investigations
Investigations